INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Dabrafenib alone — advanced melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00237
Version 6

Reference treatment cycle

Treatment stages

01 **Ongoing oral treatment**
28-day reference cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

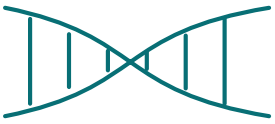
What each medicine does

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

BRAF TARGETED TREATMENT

Dabrafenib

Tafinlar



Blocks an altered BRAF signal

Dabrafenib inhibits the altered BRAF V600 protein involved in melanoma growth. This guide describes dabrafenib alone for advanced melanoma. It does not include a second targeted medicine or a postoperative one-year treatment schedule.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

A temperature of 38°C or higher requires immediate contact and interruption under the current product information; follow the written plan and do not restart without team advice.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Record where the melanoma began, its pathology result and the sites involved now. This pathway treats melanoma that cannot be fully removed or has spread. Treatment after complete removal has a different aim and a separate plan.

The tumour test

The laboratory checks a sample of the melanoma for a BRAF V600 change. The result can identify a target for certain medicines. A change found in the tumour does not, by itself, establish an inherited condition.



Scans and local treatments

Add planned scans, any operation and any radiotherapy to the same calendar. Ask what each part is intended to achieve and which team arranges it.

New changes still matter

Tell your team about a changing mole, a persistent sore, a new lump or an unexplained symptom. Improvement in a known tumour does not explain a new problem elsewhere.

Your medicines and overall plan

The schedule

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Important boundaries

This is dabrafenib monotherapy for advanced melanoma. Do not import the food, heart-testing, fertility or duration instructions of a different BRAF/MEK combination. The NCCP fever table states 38.5°C, while current EMA product information states interruption at 38°C for monotherapy; this guide uses the earlier current-product safety trigger and the team must reconcile the written plan.

Evidence and choices

Median progression-free survival was 5.1 versus 2.7 months; hazard ratio 0.30, 95% CI 0.18–0.51. These are group results from that study, not a prediction for you. Advanced-disease monotherapy evidence, not postoperative dabrafenib/trametinib evidence. The comparator and treatment era do not establish superiority over current combinations.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

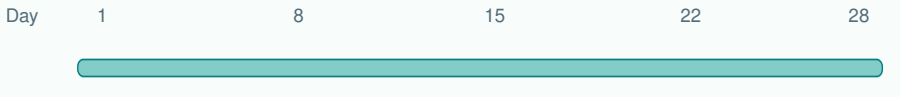
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Dabrafenib alone — advanced melanoma

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REFERENCE CYCLE

Ongoing oral treatment

Ongoing oral treatment **Repeat only as prescribed**

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00237 v6: current regimen source · EMA Tafinlar product information, updated 19 May 2026

THE WHOLE TREATMENT JOURNEY

Dabrafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00237	Confirm BRAF V600 and check blood count, kidney and liver function, electrocardiogram (ECG), medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.	Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.	If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.	Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00237	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00237	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 **Ongoing oral treatment**

28-day reference cycle

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual starting dose is 150 mg twice daily, about 12 hours apart. Take capsules at least one hour before or at least two hours after food. Swallow whole with water. Use the prescribed strength and dose, and check the label again after any interruption or dose change.

STEP 2

Missed, vomited or held treatment

Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.

Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily tablets / capsules	 **Days 1–28** Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.
Safety and symptom review	 **Days 1–28** Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.

Schedule and review: Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Holds, missed treatment and phase changes: Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Why these bands stay broad: This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00237 v6: current regimen source ↗ · EMA Tafinlar product information, updated 19 May 2026 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–28: At home each day

The usual starting dose is 150 mg twice daily, about 12 hours apart. Take capsules at least one hour before or at least two hours after food. Swallow whole with water. Use the prescribed strength and dose, and check the label again after any interruption or dose change. Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Support around the medicines

1 Supportive medicines and symptom plan

NCCP classifies dabrafenib as minimal-to-low emetogenic risk; anti-sickness treatment follows local and individual need. Agree a written fever plan and keep a thermometer and actual urgent-contact number. The team may prescribe fever treatment and hydration advice after assessment; fever medicine is not a substitute for contacting them. Anti-sickness treatment is selected individually.

2 Skin and daily care

Inspect skin and report new lumps, persistent warty growths, sores or changing moles. Report painful thickening or redness of palms and soles before it limits walking or hand use. Protect skin from the sun and attend monthly skin reviews.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during dabrafenib alone and for two weeks after the last dose. Hormonal contraception may be less reliable; agree an effective alternative. A longer interval applies if another medicine requires it, so use your actual prescribed plan. Discuss fertility and breastfeeding before treatment.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.

During treatment

Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.

Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared. Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan. Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers This is dabrafenib monotherapy for advanced melanoma. Do not import the food, heart-testing, fertility or duration instructions of a different BRAF/MEK combination. The NCCP fever table states 38.5°C, while current EMA product information states interruption at 38°C for monotherapy; this guide uses the earlier current-product safety trigger and the team must reconcile the written plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dabrafenib alone — advanced melanoma · support plan

Active medicines: Dabrafenib

Emetogenic risk: Minimal to low per NCCP 00237 v6; local policy

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies dabrafenib as minimal-to-low emetogenic risk; anti-sickness treatment follows local and individual need. Agree a written fever plan and keep a thermometer and actual urgent-contact number. The team may prescribe fever treatment and hydration advice after assessment; fever medicine is not a substitute for contacting them. Anti-sickness treatment is selected individually.
- Monthly blood count, kidney and liver tests, and monthly skin review. Repeat electrocardiogram (ECG) when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.
- If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.
- Use effective contraception during dabrafenib alone and for two weeks after the last dose. Hormonal contraception may be less reliable; agree an effective alternative. A longer interval applies if another medicine requires it, so use your actual prescribed plan. Discuss fertility and breastfeeding before treatment.
- This is dabrafenib monotherapy for advanced melanoma. Do not import the food, heart-testing, fertility or duration instructions of a different BRAF/MEK combination. The NCCP fever table states 38.5°C, while current EMA product information states interruption at 38°C for monotherapy; this guide uses the earlier current-product safety trigger and the team must reconcile the written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fever / head

Dabrafenib

Fever, chills and headache can occur. At 38°C or higher, stop and call under the current product information; severe weakness, dizziness or confusion is urgent.

2 Eyes

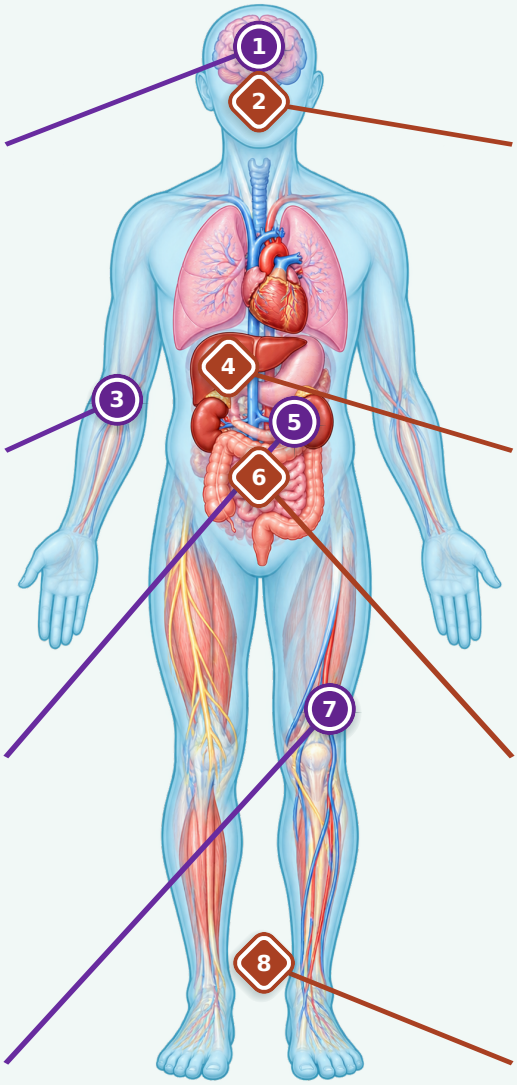
Dabrafenib

Pain, redness, light sensitivity or blurred vision may be inflammation. Prompt eye assessment may be needed.

3 Skin

Dabrafenib

Rash, thickened skin and new skin cancers can occur. Report a changing lesion, a persistent sore or severe rash.



4 Liver

Dabrafenib

Routine blood tests look for liver changes. Yellow skin or eyes or dark urine need prompt review.

5 Kidneys / hydration

Dabrafenib

Fever, vomiting or diarrhoea can cause dehydration and kidney injury. Report reduced urine or inability to drink.

6 Abdomen / pancreas

Dabrafenib

Nausea and bowel changes can occur. Severe or persistent abdominal pain needs urgent assessment because pancreatic inflammation is possible.

7 Joints / muscles

Dabrafenib

Joint pain, muscle pain or fatigue may affect activity. Report increasing pain or weakness and ask about suitable relief.

8 Hands / feet

Dabrafenib

Sore thickened skin or redness on palms and soles can affect walking and grip. Report it early and ask for skin care tailored to your symptoms.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00237 v6 supportive-care section; current product information for medicine-specific precautions**
   https://healthservice.hse.ie/documents/6726/237_Dabrafenib.pdf
2. **EMA Tafinlar product information, updated 19 May 2026 ↗**
   https://www.ema.europa.eu/en/documents/product-information/tafinlar-epar-product-information_en.pdf
3. **Hauschild et al. BREAK-3, Lancet 2012**
   https://pubmed.ncbi.nlm.nih.gov/22735384/
4. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
5. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
6. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
7. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
8. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet